

THE CLINICAL AI BRIEF™

Practical AI insights for clinicians across healthcare, education, and care settings

ISSUE 03 | THE AI SCRIBE IN YOUR CLINIC

Before We Begin: A Word of Appreciation

Thank you for the responses we have already received from our first two issues. Clinicians across disciplines are asking sharp, important questions and that is exactly the spirit this newsletter was built to hold. Keep them coming.

Today we get into something that is moving fast in clinical settings and deserves your full attention: AI documentation tools, and specifically ambient AI scribes.

The Insight: What an Ambient AI Scribe Actually Does

An ambient AI scribe is a software tool that listens to a clinical encounter in real time (a session, evaluation, visit, or consultation) and automatically generates clinical documentation from that audio. It produces a draft note, progress report, or summary that the clinician then reviews, edits, and signs off on.

These tools are being adopted at a significant pace. Some estimates suggest ambient AI scribes can reduce documentation time by 40 to 70 percent. For clinicians who spend hours each week on notes after patient contact, that is a meaningful number.

But there is more to understand before you sign on.

HOW THEY WORK -- IN PLAIN LANGUAGE

Ambient scribes use a combination of automatic speech recognition (ASR) and large language models (LLMs) -- the same underlying technology behind tools like ChatGPT. The audio of your session is transcribed, and then the LLM generates structured clinical language from that transcript based on templates it has been trained on.

What this means in practice: the tool does not understand your patient. It identifies patterns in speech and maps them to documentation conventions. The clinical reasoning that connects those words to a treatment decision is still entirely yours.

Where ambient scribes are being used right now:

- Hospital medicine and primary care (early adopters with the most market penetration).
- Outpatient rehabilitation: PT, OT, and SLP settings in private practice and health systems.
- School-based SLP documentation pilots in several large districts.
- Behavioral health and social work settings exploring note generation for session summaries.
- Telehealth platforms integrating scribes directly into the visit interface.

What Clinicians in Rehab and Allied Health Need to Know

Most ambient AI scribe tools were built and validated with physician workflows in mind specifically, physician-patient encounters in outpatient medicine. That matters for us.

Speech-language pathology, occupational therapy, physical therapy, and social work involve clinical interactions that are structurally different. Sessions may involve indirect services, consultation, family training, group treatment, or non-verbal patients. The documentation requirements are different. The coding requirements are different. And the nuance required to capture skilled intervention is significant.

A tool trained on physician-patient dialogue may not produce accurate or billable documentation for your discipline without substantial editing. That editing time has to be factored into any honest cost-benefit analysis.

Five questions to ask before your organization adopts an ambient scribe:

Was this tool validated on clinical populations and session types similar to mine?

Who owns the audio recording and the generated transcript? Where is it stored?

What is the error rate for clinical terminology specific to my discipline?

What is my liability if the auto-generated note contains a clinical inaccuracy I missed?

What opt-in or opt-out rights do my patients have regarding AI documentation of their session?

The Clinician's Role Does Not Shrink -- It Sharpens

Here is the reframe worth holding onto: ambient scribes do not reduce the importance of your clinical observation. They increase your responsibility for reviewing documentation with precision.

A note generated by an AI scribe is a first draft, not a finished product. The clinician who signs it is accountable for every word. That means your skill in recognizing what is accurate, what is missing, and what is clinically indefensible becomes more valuable, not less, in an environment where AI is generating the rough text.

Your Reflection for This Week

If an ambient scribe were recording your next session and generating your documentation, what would it miss? What would it get wrong? What would it get right?

That exercise will tell you something useful about the gap between what AI can capture and what only you can provide.

COMING UP IN ISSUE 04

We will explore AI and clinical decision support, tools that do not just document, but make recommendations. What does it mean when an algorithm suggests a diagnosis, a treatment path, or a discharge timeline? And what should clinicians know about how to evaluate those recommendations?

The conversation about AI in clinical practice needs your voice in it.

Questions or topic suggestions: newsletter@clinicalaicollective.net

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